UNICEF FEMTECH VENTURES · SECOND CALL · 2026

“Watch the clock.”

The 2-minute pitch-video script for **Tara**: an open-source, voice-first maternal care network for frontline health workers, in Kannada and Hindi.

PILLAR

1 · Healthcare access for women and girls

Supporting: Pillar 2 (AI in underserved languages) and Pillar 3 (safety)

RUNTIME

2:00

264 spoken words at ~135 words a minute

FORMAT

Act 1–4 · two-column A/V script

Problem → Solution → Live demo → Scale & ask

DEADLINE

4 November 2026

Apply through the call page’s “Apply now” form

Built from the UNICEF Venture Fund pitch playbook (4-act, 260–280 words, dual-column, explicit demo markers) and the Femtech Ventures call text. Bracketed or highlighted items are for the team to confirm before recording.

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01 The call, decoded: why Pillar 1

UNICEF Femtech Ventures backs companies "using frontier technologies to build open-source solutions for the health, wellbeing, and sexual and reproductive health and rights of girls and women": up to **US$100,000 equity-free**, 12 months of mentorship and access to UNICEF's network, including governments.

The three pillars, and where Tara stands

Pillar	What the call asks for	Tara's fit
1 · Healthcare access for women and girls PRIMARY	"The continuum of care, from antenatal and postnatal support…" · point-of-care tools with **automated interpretation** for frontline health workers · platforms **linking screening, referral and follow-up** · continuity of care "from conception through the first 1,000 days", including **remote care in low-bandwidth environments** · tools covering community health worker workload.	Exact match. A 24-question antenatal check-in, interpreted automatically by auditable rules, linked to the doctor's referral and the mother's follow-up, on a basic phone and a weak network.
2 · Informed, safe and accessible reproductive health SUPPORTING	"AI-powered accessible platforms… in **underserved languages**, especially for low-literacy and low-bandwidth environments" · CHW platforms for case management, referral and follow-up · consent- and privacy-preserving data.	Sakhi is a voice AI in Kannada and Hindi: no typing, no English, no reading required.
3 · Mental wellbeing, safety and protection SUPPORTING	Decision support for CHWs on risk identification and specialist referral · tools for frontline workers.	Risk flags + referral for the ASHA, and an SOS that reaches the nearest on-duty ASHAs.

> **Positioning line (use it everywhere: video, form, deck):**
> Tara is a Pillar 1 point-of-care platform: a voice AI that lets frontline health workers screen pregnant women in their own language, and links that screening to referral and follow-up, even on low bandwidth.

The call's own words, the script mirrors them

Call language	Where it is heard or seen in the film
frontier technologies · generative and agentic AI	Act 2: "Its AI companion, Sakhi…"; Act 3 shows the AI conducting the check-in
open-source licensing and principles	Act 2 "open-source"; Act 4 "Tara is open source" + repository on screen
automated interpretation · frontline health workers	Act 3: speech → text → rules flag severe pre-eclampsia "on the spot"
screening, referral and follow-up	Act 3: ASHA screening → doctor's one-tap referral → advice on the mother's phone

underserved languages · low literacy	Kannada on screen with subtitles; “in her own words”, “in her own language”
low-bandwidth · low-resource settings	Act 2 badge “basic Android phone”; Act 4 “runs on low bandwidth and keeps working offline”
safety-by-design · consent · privacy	Act 4: “safe by design: consent first, minimal data, and auditable rules”
hyperlocal · designed within communities	Bidar named on screen; the ASHA and the mother are the heroes, not the app

Eligibility, as the call states it

✓ Working with a functional prototype — Tara runs today (ASHA, doctor and mother apps; voice check-in; SOS).

! Committed to open-source licensing — **add a LICENSE file to the repository** (none exists yet) before you submit or show the repo.

! Registered in a UNICEF programme country — confirm India on the programme-country list linked from the call page, and that the company is registered.

✓ Safety-by-design is a condition of investment — the film states it; be ready to show consent screens and data handling in the application.

i Women-led startups and founders under 35 are strongly encouraged — if that is you, say it in the form (and optionally in Act 4’s on-screen text).

02 The creative idea: a film that is its own proof

Most pitch videos *describe* speed. This one *shows* it. A small timer starts at 00:00 in the first frame and runs in the corner for the whole film. In that time the audience watches **one** danger sign, Lakshmi's blood pressure, travel from a village register to a doctor and back to her phone. At 1:30 the timer freezes on the SOS screen. The film's runtime *is* the argument.

One mother, not a market

UNICEF's strongest demos are child- or facility-specific. Ours is mother-specific: Lakshmi D., 7 months, BP 148/96. Every screen in the demo is about her.

Three phones, one story

ASHA → doctor → mother. Each cut changes phone and colour of the UI, so a viewer with the sound off still follows the journey.

The ASHA is the hero

Sakhi helps; Sunita decides and acts. This answers "who uses it?" in the first 25 seconds and fits the call's "designed within communities".

Proof over promise

Every demo beat is a real screen from the working prototype, with real Kannada audio and real rule codes. No mock-ups in Act 3.

The emotional line, in four beats

Beat	The viewer feels	Carried by
Act 1	Unease: the warning is real and written down, but it stays in a notebook.	"That warning can wait days for a doctor."
Act 2	Relief: something simple, in her language, already in her hand.	Logo resolve; three badges
Act 3	Momentum, then safety: each tap moves Lakshmi closer to care.	Fast cuts, timer, SOS radar
Act 4	Confidence: it is open, safe and ready to scale with UNICEF.	"Every danger sign, heard in time."

Brand line

Every danger sign, heard in time.

Used once, as the last spoken words, and on the end card. Do not use it earlier: the film earns it.

03 What UNICEF's best demos do, and how Tara does it

The playbook's analysis of shortlisted pitches (ClimaSalud, ClimaSense, EAGLE AI, SCP ChildShield) found four things every strong demo shows. The script hits all four, on screen, in Act 3.

Winning signal	How they showed it	How Tara shows it (time)
1 · Specificity (one child / one facility)	A named school on a risk map	One named mother, one reading: Lakshmi D., BP 148/96 (0:07 onward)
2 · Real-time automated alerts	Alerts in under 30 s	Rule PE-04 fires as she answers; the case is first in the doctor's queue (0:58–1:20)
3 · Low-bandwidth / offline access	SMS, WhatsApp, USSD, basic phones	Voice on a basic Android phone, low bandwidth, keeps working offline (0:35, 1:30)
4 · Open-source sustainability	Public repository, adaptable by governments	"Tara is open source" with the repository on screen (1:30)

> **And what they avoided:** tech-stack jargon. The script never says WebSocket, VAD, LiveKit or STT. It says "her words become text in about a second" and "one tap confirms the referral": people doing things.

Closest benchmark: EAGLE AI (Kenya)

Voice-first AI in local languages, connected to real workflows. Tara goes one step further, and the film must make the difference visible: **the voice does not end in a chat**. It ends in a clinical rule, a doctor's referral and a mother's phone. That closed loop, screening → referral → follow-up, is literally a Pillar 1 bullet.

04 The 4-act plan and word budget

Problem → Solution → Live demo → Scale & ask, as the playbook prescribes. Word counts below are computed from the script itself.

Act	Time	Purpose	Budget	Script
Act 1 Problem	0:00 – 0:25	Emotional hook and a mother-level risk, stated plainly.	**55–60**	**60**
Act 2 Solution	0:25 – 0:45	What Tara is, in one breath, and who it serves.	**45–50**	**49**
Act 3 Live demo	0:45 – 1:30	One danger sign, followed across three phones.	**90–100**	**100**
Act 4 Scale & ask	1:30 – 2:00	Open source, low bandwidth, safety by design, the ask.	**50–55**	**55**
Total spoken words (presenter only; Sakhi's app audio is extra)			**260–280**	**264**

Pace

264 words in 120 seconds is about 132 words a minute: the playbook's 130–140 band, with breathing room for Sakhi's two Kannada lines (about 4 seconds) and the frozen timer. Read slightly slower in Act 1 and faster in Act 3.

Who speaks when

- **Founder (VO)**: Acts 1 and 3, over footage and screen recordings.
- **Founder (on camera)**: Acts 2 and 4. Mid-shot, eye level, the phone in hand in Act 2.
- **Sakhi (app audio)**: the real app voice in Act 3, never re-voiced. Duck the VO under it.
- **Sunita (ASHA)**: one real spoken answer, "ಬಿ ಪಿ 148 ಬೈ 96", recorded with consent.

05 The master script

Left: what is on screen, with explicit demo markers ([TAP], [ZOOM INTO], [PRESS]) for the editor. Right: every spoken word. Record the screen first, then read the voice-over against it.

ACT 1 · PROBLEM · 0:00 – 0:25 · 60 words

Time	Visual & demo cues (on screen)	Spoken script
0:00 – 0:07 COLD OPEN	Black frame. A small white timer appears top-right at 00:00 and starts running (it runs for the whole film). Fade up: dawn, a village lane in Bidar. An ASHA worker walks with a cloth bag and a paper register. Lower third: "Bidar district, Karnataka, India"	FOUNDER (VO) "Watch the clock in the corner. Before this film ends, one danger sign will travel from a village to a doctor." 21 words
0:07 – 0:25 PROBLEM	Close-up: a handwritten register line "BP 148/96 — headache, blurred vision". Text builds over it: HIGH BP + HEADACHE + BLURRED VISION = SEVERE PRE-ECLAMPSIA RISK. The register closes. On-screen: "The warning exists. It just doesn't travel."	FOUNDER (VO) "Sunita is an ASHA worker. Her mothers live across three villages; their records live on paper. Lakshmi's blood pressure reads one forty-eight over ninety-six, with headache and blurred vision: severe pre-eclampsia. That warning can wait days for a doctor." 39 words

ACT 2 · SOLUTION · 0:25 – 0:45 · 49 words

Time	Visual & demo cues (on screen)	Spoken script
0:25 – 0:35 SOLUTION	Logo resolve: Tara leaf grows into the wordmark. Match-cut to Sunita's phone: the ASHA home screen (villages, "Today's visits"). Super: "Tara — open-source maternal care network"	FOUNDER (ON CAMERA) "This is Tara: an open-source, voice-first maternal care network, built for the frontline health workers who already reach every mother." 20 words
0:35 – 0:45 SOLUTION	Sakhi's orb pulses on screen. Three badges pop in, one per beat: KANNADA · HINDI | NO TYPING | BASIC ANDROID PHONE. Badges as above	FOUNDER (ON CAMERA) "Its AI companion, Sakhi, talks with the ASHA in Kannada or Hindi, with no typing, and turns a spoken check-in into a screened clinical report the doctor can trust." 29 words

ACT 3 · LIVE DEMO · 0:45 – 1:30 · 100 words		
Time	**Visual & demo cues (on screen)**	**Spoken script**
0:45 – 0:58 **[DEMO START]**	Screen recording, ASHA phone: [TAP "Sakhi Direct"] → [TAP "Lakshmi D."] → [TAP "Start questions"]. Sakhi speaks; subtitle shows her Kannada line with English underneath. Sunita answers aloud. Duck the VO under Sakhi's line (~2 s). Sakhi: "ತಾಯಿಯ ಇಂದಿನ ಬಿಪಿ ಎಷ್ಟು ದಾಖಲಾಗಿದೆ?" (What is her BP today?) · Sunita: "ಬಿ ಪಿ 148 ಬೈ 96"	**FOUNDER (VO) + SAKHI (APP AUDIO)** "Sakhi asks one question at a time, in a warm, familiar voice, and Sunita simply answers in her own words." 20 words
0:58 – 1:10 **[DEMO FEATURE]**	The answer bubble fills: "ಬಿ ಪಿ 148 ಬೈ 96" → "148/96 mmHg". A red danger card slides in. [ZOOM INTO DANGER CARD] → [TAP "Why this risk"] shows the rule audit: PE-01 AND PE-03 → PE-04. Super: "Speech → text ≈ 1 s · rule PE-04 fired"	**FOUNDER (VO)** "Her spoken words become text in about a second. Transparent clinical rules, not a black box, flag severe pre-eclampsia on the spot, and every rule can be audited." 28 words
1:10 – 1:20 **[DEMO ACTION]**	Cut to the doctor's phone, home "Today": Lakshmi D. at the top of "Needs you", tagged URGENT. [TAP CASE] → summary banner + vitals (BP HIGH) → [TAP "Confirm referral"]. Haptic flash on the button. Super: "Doctor: 1 tap"	**FOUNDER (VO)** "When the check-in ends, Lakshmi is already first in the doctor's queue, with her vitals and the whole conversation. One tap confirms the referral." 24 words
1:20 – 1:30 **[DEMO ACTION]**	Cut to Lakshmi's phone: the doctor's advice arrives in Kannada. Then [PRESS SOS]: the screen turns red, ripples, "Reporting ASHA workers nearby", a radar sweep, then "Sunita · 1.2 km · On the way". The corner timer FREEZES. Timer freezes · Super: "Village → doctor → mother, in this demo: 01:30"	**FOUNDER (VO)** "Clear, simple advice reaches Lakshmi in her own language. And if she is ever in danger at home, one SOS alerts the nearest ASHAs, her own ASHA first." 28 words

ACT 4 · SCALE & ASK · 1:30 – 2:00 · 55 words		
Time	**Visual & demo cues (on screen)**	**Spoken script**
1:30 – 1:45 **SCALE**	Motion graphic, three columns building left to right: OPEN SOURCE (repository page) | LOW BANDWIDTH · WORKS OFFLINE | SAFETY BY DESIGN (consent, minimal data, auditable rules). Three column titles as above	**FOUNDER (ON CAMERA)** "Tara is fully open source. It runs on low bandwidth and keeps working offline, and it is safe by design: consent first, minimal data, and auditable rules." 27 words
1:45 – 2:00 **ASK**	Sunita and Lakshmi (consented footage or illustration) on a doorstep, phone between them. End card: Tara logo, "Every danger sign, heard in time.", "Applying to UNICEF Femtech Ventures · Pillar 1: Healthcare access for women and girls". End card text as above (no UNICEF logo unless UNICEF has approved its use)	**FOUNDER (ON CAMERA)** "With UNICEF Femtech Ventures, we will pilot with two hundred ASHA workers in Bidar district, then open Sakhi to every Indian language. Every danger sign, heard in time." 28 words

06 Read-through script (presenter copy)

The spoken words only, act by act, for rehearsal and the teleprompter. 264 words.

Act 1 · Problem 0:00 – 0:25

Watch the clock in the corner. Before this film ends, one danger sign will travel from a village to a doctor. Sunita is an ASHA worker. Her mothers live across three villages; their records live on paper. Lakshmi's blood pressure reads one forty-eight over ninety-six, with headache and blurred vision: severe pre-eclampsia. That warning can wait days for a doctor.

Act 2 · Solution 0:25 – 0:45

This is Tara: an open-source, voice-first maternal care network, built for the frontline health workers who already reach every mother. Its AI companion, Sakhi, talks with the ASHA in Kannada or Hindi, with no typing, and turns a spoken check-in into a screened clinical report the doctor can trust.

Act 3 · Live demo 0:45 – 1:30

Sakhi asks one question at a time, in a warm, familiar voice, and Sunita simply answers in her own words. Her spoken words become text in about a second. Transparent clinical rules, not a black box, flag severe pre-eclampsia on the spot, and every rule can be audited. When the check-in ends, Lakshmi is already first in the doctor's queue, with her vitals and the whole conversation. One tap confirms the referral. Clear, simple advice reaches Lakshmi in her own language. And if she is ever in danger at home, one SOS alerts the nearest ASHAs, her own ASHA first.

Act 4 · Scale & ask 1:30 – 2:00

Tara is fully open source. It runs on low bandwidth and keeps working offline, and it is safe by design: consent first, minimal data, and auditable rules. With UNICEF Femtech Ventures, we will pilot with two hundred ASHA workers in Bidar district, then open Sakhi to every Indian language. Every danger sign, heard in time.

Delivery notes

- Say numbers the way you would to a friend: “one forty-eight over ninety-six”.
- Pause one beat after “That warning can wait days for a doctor.” Let the register close in silence.
- “ASHA”: say it as a word (AA-sha), never as letters.
- Smile on “Every danger sign, heard in time.” and hold eye contact for two seconds after it.

07 Shot list and screen-recording runbook

Act 3 must be the real app. Record each take on a clean demo build, at 1080 × 1920, with Do Not Disturb on. Reset demo data first (Profile → Reset demo data) so Lakshmi's record matches the script.

Shot	Phone / role	Exact steps (taps)	Must be visible
S1	—	Live action: village lane at dawn; ASHA walking with register. 6–8 s, handheld, natural light.	No identifiable patient faces without consent
S2	—	Macro: register page with "BP 148/96, headache, blurred vision" (a prop written for the film).	Handwriting legible for 2 s
S3	ASHA	Open app as ASHA → home screen. Slow scroll to show villages and "Today's visits".	Kannada UI, Sunita's name
S4	ASHA	Home → **Sakhi Direct** → **Lakshmi D.** → wait for the green "connected" line → **Start questions**. Let Sakhi greet; skip ahead (Next) to the BP question; answer aloud "ಬಿ ಪಿ 148 ಬೈ 96".	Sakhi orb, Kannada subtitle, the answer bubble
S5	ASHA	Hold on the red danger card; open "Why this risk".	PE-01 · PE-03 · PE-04 lines
S6	Doctor	Switch role to Doctor → home "Today" → Lakshmi D. (URGENT) → case page → **Confirm referral**.	"Needs you", BP HIGH tile, the button press
S7	Mother	Switch role to Mother (Lakshmi) → the doctor's advice notification → open it.	Kannada advice text
S8	Mother	Tap the red **SOS** tab on the right edge (or press the side keys up, down, up, down) → let the countdown run → "Reporting ASHA workers nearby" → wait for "On the way".	Red screen, radar, Sunita · km · On the way
S9	—	Screen capture of the public repository page (README + LICENSE visible).	"open source" licence name
S10	—	Founder on camera: Act 2 and Act 4, two takes each, one with phone in hand.	Eye level, quiet room

> **Editing rule:** cut on the tap. Each [TAP] marker in the script is a cut point; trim the waiting before each tap so Act 3 fits 45 seconds without speeding up the voice. Never speed up or fake Sakhi's audio.

08 On-screen text, subtitles and audio

Burned-in subtitles (always on: many viewers watch muted)

Time	Original	English subtitle
0:47	ತಾಯಿಯ ಇಂದಿನ ಬಿಪಿ ಎಷ್ಟು ದಾಖಲಾಗಿದೆ?	Sakhi: “What is the mother’s blood pressure today?”
0:51	ಬಿ ಪಿ 148 ಬೈ 96	Sunita: “BP 148 by 96.”
1:22	ಇಂದೇ PHC ಗೆ ಬನ್ನಿ	Advice: “Come to the PHC today.” (use the exact text the app shows)

Supers and lower thirds

Time	Text
0:00–2:00	Running timer, top right, white, tabular figures: 00:00 → freezes at 01:30
0:02	Bidar district, Karnataka, India
0:15	HIGH BP + HEADACHE + BLURRED VISION = SEVERE PRE-ECLAMPSIA RISK
0:23	The warning exists. It just doesn’t travel.
0:28	Tara — open-source maternal care network
0:38	KANNADA · HINDI | NO TYPING | BASIC ANDROID PHONE
1:02	Speech → text ≈ 1 s · rule PE-04 fired
1:15	Doctor: 1 tap
1:28	Village → doctor → mother, in this demo: 01:30
1:33	OPEN SOURCE | LOW BANDWIDTH · WORKS OFFLINE | SAFETY BY DESIGN
1:50	Every danger sign, heard in time. · Applying to UNICEF Femtech Ventures · Pillar 1

Sound

- Music: a soft, warm bed (acoustic, no vocals) at −24 LUFS under voice; it drops out for Sakhi’s first line, returns at the logo.
- Act 3: a single soft tick on each [TAP]; one low pulse when the danger card appears; the SOS ripple has its own sound.
- Final mix: −14 LUFS integrated, voice always intelligible on a phone speaker.

09 Production, safeguarding and delivery specs

Safeguarding (a condition of UNICEF's investment, so it is also in the film)

- ✓ All data on screen is synthetic demo data (Lakshmi D., Sunita). Say so in the description: "All patient data shown is fictional."
- ✓ Anyone filmed signs a consent and release form, in Kannada, read aloud to them. No minors on camera.
- ✓ No real patient registers, faces of patients, phone numbers or addresses anywhere in frame.
- ✓ The SOS sequence is labelled as a prototype demonstration; it must not suggest it replaces 108 emergency services.
- ! Do not use the UNICEF logo on the end card unless UNICEF has approved it; use the text line instead.

Delivery

Item	Spec
Master	1920 × 1080, 25 or 30 fps, H.264 MP4, ≤ 2:00, stereo AAC 48 kHz
Vertical cut	1080 × 1920 for social / mobile reviewers: same audio, phone recordings full screen
Captions	Burned-in English + a separate .srt file
Hosting	Unlisted YouTube or Vimeo link (works on low bandwidth, reviewers can lower quality)
File name	Tara_UNICEF_Femtech_Pitch_2026.mp4

Schedule to the deadline (4 November 2026)

When	What
Day 1	Confirm placeholders (pilot size, district, licence), add LICENSE, reset demo data, rehearse script
Day 2	Screen recordings S3–S9; on-camera S10
Day 3	Field footage S1–S2 with consent; voice-over recording
Day 4	Edit, subtitles, mix; internal review against the fact-check sheet
Day 5	Final export; submit with at least 48 hours to spare

10 Fact-check sheet: say only what is true

Reviewers check claims. Every factual statement in the film is listed here with its status. Change the script, not the facts.

Claim in the film	Status	Basis / what to do
Sakhi talks in Kannada or Hindi	**True (prototype)**	Both languages in the check-in today.
Asks one question at a time; 24 antenatal questions	**True**	The check-in engine asks the 24 questions in order, one per turn.
Her words become text in about a second	**True (measured)**	Speech-to-text measured at about 0.4–1.2 s per answer in testing.
Transparent, auditable rules flag severe pre-eclampsia	**True**	Deterministic rules PE-01/PE-03 → PE-04, shown in the rule audit.
First in the doctor's queue; one tap confirms the referral	**True**	Doctor home lists urgent cases first; "Confirm referral" sends the advice.
Advice reaches the mother in her language	**Check**	Make sure the advice shown on the mother's phone in the take is in Kannada.
SOS alerts the nearest ASHAs, her own first	**Prototype**	Works in the app as a demonstration with demo locations; label it as such.
Runs on low bandwidth; keeps working offline	**True**	Answers are kept on the phone and sent when the network returns.
Open source	**Action**	Add a LICENSE (for example Apache-2.0 or MIT) and make the repository public before submitting.
Consent first, minimal data	**Design claim**	Have the consent flow and data policy ready to show in the application.
Pilot with two hundred ASHAs in Bidar	**Placeholder**	Replace with the team's real plan before recording (number and district).

> **Kept out on purpose:** national maternal-mortality statistics. They are powerful but easy to misquote. If you add one, take it from the latest WHO or Government of India source and cite it on screen.

11 Submission checklist

- ☐ Placeholders confirmed: pilot size, district, licence name
- ☐ LICENSE file added; repository public; README explains how to run Tara
- ☐ Company registration in a UNICEF programme country confirmed (India on the list)
- ☐ Demo data reset; every Act 3 screen recorded from the real app
- ☐ Consent forms signed for everyone on camera
- ☐ Film ≤ 2:00; spoken words within 260–280; burned-in subtitles
- ☐ Fact-check sheet signed off by two people
- ☐ No UNICEF logo unless approved
- ☐ Unlisted link tested on a phone over mobile data
- ☐ Application submitted through the call page before 4 November 2026

Every danger sign, heard in time.